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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

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SAMPLE : Serum

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Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
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P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

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~~End of report~~

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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

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~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

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REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **TARANNUM**
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Reg No : **HL4801832378 / RAJ52**
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Age : 48 Year | **Sex :** Female
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SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

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~~End of report~~

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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

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REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

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~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
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Dr. MEENU RANI (MD PATH)
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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

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NOTE :

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~~End of report~~

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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	**28.0**	ng/mL	30 - 100

Reference Range
Deficiency < 20 ng/mL
Insufficiency 20– 30 ng/mL
Sufficiency 30–100 ng/mL
Toxicity :>100 ng/mL

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.
Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MRS. PRATIBHA SADANAND JOSHI**	**Age :** 55 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** OM LAB PACHORA
Ref By	: **DR. DR. AJAYSING PARDESHI SIR (M.D.)**	**Registered on :** 08-08-2026 10:14 PM	
Reg No	: **HL1701807513 / MH130**	**Received on** : 09-08-2026 02:13 PM	
Barcode	: **HEB106819**	**Reported on** : 09-08-2026 11:45 PM	**SAMPLE :** Body Fluid
INV	: BODY FLUID FOR GRAM STAIN		

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	. - .
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **ELIYAS SHAIKH**	**Age :** 62 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** NAGRIK DIAGNOSTICS
Ref By	: **DR. CENTRAL HOSPITAL**	**Registered on :** 29-08-2026 11:36 AM	
Reg No	: **HL4901829960 / RAJ02**	**Received on** : 29-08-2026 06:58 PM	
Barcode	: **HV027093**	**Reported on** : 06-09-2026 12:17 AM	**SAMPLE :** SLIDE
INV	: FNAC (Fine Needle Aspiration Cytology)		

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SHRIRANG SANGORAM**
Ref By : **DR. SELF**
Reg No : **HL4901832195 / MH026**
Barcode : **396709455**
INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | **Sex :** Male
Registered on : 31-08-2026 02:59 PM
Received on : 01-09-2026 04:44 PM
Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	**3000**	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-
Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Comments
Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SHABNAM**
Ref By : **DR. SELF**
Reg No : **HL6301832464 / MH019**
Barcode : **Hei087746**
INV : Insulin Fasting

Age : 52 Year | **Sex :** Female
Registered on : 31-08-2026 05:25 PM
Received on : 31-08-2026 09:21 PM
Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING Method: CLIA	5.72	u/ml	3.0 - 25.0

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,
Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism
Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MONIKA GHODSE**	**Age :** 61 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 06:55 PM	
Reg No	: **HL5001830586 / RAJ53**	**Received on** : 29-08-2026 07:05 PM	
Barcode	: **HEN143402**	**Reported on** : 29-08-2026 09:01 PM	**SAMPLE :** Serum
INV	: Insulin - Post Prandial		

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MR. SANJAY VAZE**
Ref By : **DR. PARAS DEDHIA**
Reg No : **HL7801829210 / HYMH/BSR/H9102**
Barcode : **HED012024**
INV : Ionized Calcium

Age : 48 Year | **Sex :** Male
Registered on : 28-08-2026 12:57 PM
Received on : 28-08-2026 05:26 PM
Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :
REDCHECK LABS

SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :
Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125362**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:15 PM
Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SUCHI MARFATIA**
Ref By : **DR. SELF**
Reg No : **HL6601832626 / HYMH/KDV/H7275**
Barcode : **HEN172678**
INV : Lipase

Age : 58 Year | **Sex :** Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycrol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **DEEKSHA VARSHNEY (509133)**
Ref By : **DR. PR HOSPITAL**
Reg No : **HL7001832408 / MH300**
Barcode : **HEJ072990**
INV : Luteinising Hormone (LH)

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 04:45 PM
Received on : 01-09-2026 07:58 AM
Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating

follicular phase	1.9–12.5
midcycle peak	8.7–76.3
luteal phase	0.5–16.9
Pregnant	< 0.1–1.5
Postmenopausal	15.9–54.0
Contraceptives	0.7–5.6

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.1,2
LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.3,4 In males, LH is also called interstitial cell-stimulating hormone (ICSH).1,2 In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.5-7 LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.3 Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **TARANNUM**
Ref By : **DR. YUSUF KHAN**
Reg No : **HL4801832378 / RAJ52**
Barcode : **HEN112154**
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | **Sex :** Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :

Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.

While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **GANESH DUBAL**
Ref By : **DR. BALLAL**
Reg No : **HL2501827161 / RAJ05**
Barcode : **HED104491**
INV : Microalbumin 24 Hrs URINE

Age : 33 Year | **Sex :** Male
Registered on : 26-08-2026 01:32 PM
Received on : 26-08-2026 05:14 PM
Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :

SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	Ml/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	**53.89**	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MR. AVTAR SINGH KATTAURA**
Ref By : **DR. LAVANYA**
Reg No : **HL8801832380 / HYMH/MH642**
Barcode : **HEM072694**
INV : health Check Extended Male

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 04:30 PM
Received on : 31-08-2026 05:39 PM
Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :
PROSPERITY DIAGNOSTICS (SAMEER)

SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MRS SANGITA BAPU GAWALI**	**Age :** 48 Year | **Sex :** Female		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. YASHRAJ LAB**	**Registered on**	: 28-08-2026 06:55 PM	BHUSHAN BANKAR	
Reg No	: **HL1401829575 / MH004/008**	**Received on**	: 29-08-2026 01:16 AM		
Barcode	: **HEJ088961**	**Reported on**	: 30-08-2026 01:01 PM		
INV	: PAP Smear			**SAMPLE :** SLIDE	

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MISS .SNEHA YADAV**
Ref By : **DR. DR.SAMIR KAPADIA**
Reg No : **HL9501831765 / MH777**
Barcode : **HEN102616**
INV : Complete Blood Count (CBC) +PS+ESR

Age : 11 Year | **Sex :** Female
Registered on : 31-08-2026 10:18 AM
Received on : 31-08-2026 04:27 PM
Reported on : 01-09-2026 11:07 AM

SAMPLE COLLECTED AT :
SAMIT DIAGNOSTIC CENTER

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. AAYESHA KHATOON CHOUDHARY**	**Age :** 50 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SUCCESS DIAGNOSTIC CENTRE
Ref By	: **DR. VARUN BHATIA**	**Registered on :** 26-08-2026 11:37 AM	
Reg No	: **HL7201826910 / MH549**	**Received on** : 26-08-2026 04:49 PM	
Barcode	: **HEJ042813**	**Reported on** : 26-08-2026 05:29 PM	
INV	: Potassium		**SAMPLE :** Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	**3.66**	mEq/L	3.8 - 5.4

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **SHIVAJI SHELAR**
Ref By : **DR. SELF SELF**
Reg No : **HL2201822408 / CH002/H9588**
Barcode : **HDL080575**
INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | **Sex :** Male
Registered on : 22-08-2026 11:28 AM
Received on : 22-08-2026 11:34 AM
Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :
SEVA LAB PACHORA

SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. KUNAL BORHADE**
Ref By : **DR. J.S.MOJITYA**
Reg No : **HL4901832488 / MH520**
Barcode : **HEI011182**
INV : Prothrombin Time (PT) With INR

Age : 33 Year | **Sex :** Male
Registered on : 31-08-2026 05:45 PM
Received on : 31-08-2026 07:05 PM
Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :

SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	**1.17**	.	0.90 - 1.10
index Method: cu	85.44	%	- - -

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SUMIT AGARWAL**
Ref By : **DR. DR.NAVNEET GUPTA**
Reg No : **HL5701831905 / GUJ01**
Barcode : **GO224636**
INV : HIV 1 & 2 Antibody -Rapid

Age : 48 Year | **Sex :** Male
Registered on : 31-08-2026 12:05 PM
Received on : 01-09-2026 10:01 AM
Reported on : 01-09-2026 01:09 PM

SAMPLE COLLECTED AT :
ARYA DIGNOSTIC LAB

SAMPLE : Serum

HIV 1 & 2 ANTIBODY -RAPID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HIV -1 , Rapid Method: rapid	NON REACTIVE	.	. - .
HIV -2 , Rapid Method: rapid	NON REACTIVE	-	- - -

INTERPRETATION
Two type of human deficiency viruses, called HIV-1 and HIV-2, have been identified to date.Various subtypes of the known HI-Viruses have been described, each of which has a different geographical distribution.HIV-1 can be divided into 3 distantly related groups :group M(for main),group N (for non-M, non-O) and group O (for outlier).Based on their gentic relationship,at least 9 different subtypes (A to D, F to H,J,K) have been identifiedwithin HIV-1 group M.Antibodied to HIV protiens, indicating the presence of an HIV infection, can be found in the serum usually 6-12 weeks after infection.Due to differences in the sequence of immunodominant epitopes, especiallt in the envelope protiens HIV-1 group M HIV-1 group O and HIV-2,specific antigens are necessary to aviod failure in the detection of an HIV infection by immunoassays.

Repeatedly reactive samples must be confirmed according to recommended confirm-atory algorithms.
Confirmatory tests include Western Blot HIV RNA tests.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. AAYUSH YADAV**
Ref By : **DR. DR. SANTOSH YADAV**
Reg No : **HL2501832638 / HYMH/KDV/MATOSHREE**
Barcode : **HDM002404**
INV : Triple H - (HIV,HBsAg,HCV Antibody) ELISA

Age : 23 Year | **Sex :** Male
Registered on : 02-09-2026 04:56 PM
Received on : 02-09-2026 06:37 PM
Reported on : 03-09-2026 12:21 AM

SAMPLE COLLECTED AT :
MATOSHREE DIAGNOSTICS

SAMPLE : Serum

HIV 1 & 2 ANTIBODY ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HIV -1-2 Method: ELISA	0.12	ODRatio	0.0 - 0.90

Reference Range
Negative : < = 0.90
Equivocal : 0.91-1.10
Positive : > = 1.11

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS SABIRA SHAH**	**Age :** 27 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. NISHAT AFRIN KHAN**	**Registered on** : 31-08-2026 12:35 PM	
Reg No	: **HL3901831934 / MH771**	**Received on** : 31-08-2026 04:56 PM	
Barcode	: **HEN154194**	**Reported on** : 31-08-2026 05:32 PM	**SAMPLE :** Serum
INV	: Hepatitis C Virus (HCV Rapid)		

HEPATITIS C VIRUS (HCV RAPID)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS C VIRUS (HCV RAPID) Method: .	NON REACTIVE	.	. - .

interpretation

- Hepatitis C is a viral disease triggered by an infection of the hepatitis C virus (HCV). If not recognized or treated, it can escalate into a chronic condition causing inflammation and possible liver damage. There are often no symptoms, so hepatitis C testing is critical. Screening will help determine HCV from other hepatitis causes, including hepatitis A and hepatitis B.

  An HCV antibody test can detect if you have been infected. It looks for antibodies to HCV in the blood.

  If you are 18 or older, it is a good idea to get screened at least once for Hepatitis C. Testing is also recommended during pregnancy or if you have risk factors for HCV infection. Because acute hepatitis C often has no symptoms, screening is an important way to diagnose hepatitis C before it causes liver damage.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. AAYUSH YADAV**
Ref By : **DR. DR. SANTOSH YADAV**
Reg No : **HL2501832638 / HYMH/KDV/MATOSHREE**
Barcode : **HDM002404**
INV : Triple H - (HIV,HBsAg,HCV Antibody) ELISA

Age : 23 Year | **Sex :** Male
Registered on : 02-09-2026 04:56 PM
Received on : 02-09-2026 06:37 PM
Reported on : 02-09-2026 10:07 PM

SAMPLE COLLECTED AT :
MATOSHREE DIAGNOSTICS

SAMPLE : Serum

HEPATITIS C VIRUS (HCV) ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS C ANTIBODY Method: ELISA	0.155	Index	0 - 1.00

NOTE-

1-False positive results are seen in Autoimmune diseases, Rheumatoid factor, Hypergammaglobulinemia, Paraproteinemia, passive antibody transfer, Anti- idiotypes & Anti superoxide dismutase

2. False negative results are seen in early Acute infection, Immunosuppression & Immuno-incompetence

3. HCV RNA PCR recommended in all Reactive results to differentiate between past and present infection

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR IMRAN PATEL**
Ref By : **DR. RAHUL T CHAKOR**
Reg No : **HL6001831923 / MH593**
Barcode : **HEN116971**
INV : Hepatitis B Surface Antigen (HBs Ag) (Rapid)

Age : 46 Year | **Sex :** Male
Registered on : 31-08-2026 12:09 PM
Received on : 31-08-2026 01:28 PM
Reported on : 31-08-2026 02:39 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

HEPATITIS B SURFACE ANTIGEN (HBS AG) (RAPID)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Hepatitis B Surface Antigen (HBs Ag) ,Rapid Method: Immuno Chromatography	NEGATIVE	.	. - .

Interpretation :

HBsAg (Hepatitis B surface antigen) - A "positive" or "reactive" HBsAg test result means that the person is infected with hepatitis B. This test can detect the actual presence of the hepatitis B virus (called the “surface antigen”) in your blood. If a person tests “positive,” then further testing is needed to determine if this is a new “acute” infection or a “chronic” hepatitis B infection. A positive HBsAg test result means that you are infected and can spread the hepatitis B virus to others through your blood.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR. AAYUSH YADAV**
Ref By : **DR. DR. SANTOSH YADAV**
Reg No : **HL2501832638 / HYMH/KDV/MATOSHREE**
Barcode : **HDM002404**
INV : Triple H - (HIV,HBsAg,HCV Antibody) ELISA

Age : 23 Year | **Sex :** Male
Registered on : 02-09-2026 04:56 PM
Received on : 02-09-2026 06:37 PM
Reported on : 03-09-2026 12:21 AM

SAMPLE COLLECTED AT :
MATOSHREE DIAGNOSTICS

SAMPLE : Serum

HEPATITIS B SURFACE ANTIGEN (HBSAG)-ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Hepatitis B Surface Antigen (HBsAg) Method: ELISA	0.16	Ratio	0 - 0.90

Reference Range
Negative : < 0.90
Borderline : 0.9 - 1.1
Positive : > 1.1

Clinical significance :
Classification of a hepatitis B infection requires the identification of several serological markers of the infection. The first marker to appear during the incubation phase is HBsAg, and indicates infection with HBV. Antibodies to HBsAg generally appears after HBsAg has been cleared from the blood stream, usually 6 months after infection, and its presence represents recovery and immunity. The presence of HBsAg antibodies should not be used as the sole marker in determining a prior hepatitis b infection. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, vaccination history, clinical examination and other findings.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **ANSH RAJPUT**
Ref By : **DR. OM HOSPITAL**
Reg No : **HL9201829645 / MH244**
Barcode : **HEI047425**
INV : Hepatitis A Virus Total Antibodies (HAV-Total)

Age : 6 Year | **Sex :** Male
Registered on : 28-08-2026 08:47 PM
Received on : 29-08-2026 08:27 AM
Reported on : 29-08-2026 10:47 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

HEPATITIS A VIRUS TOTAL ANTIBODIES (HAV-TOTAL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HAV TOTAL Method: ELISA	0.45	Index	0 - 0.90

Reference Range
Negative : < 0.90
Borderline : 0.9 - 1.1
Positive : >= 1.1

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SHUBHAM DEVANE**
Ref By : **DR. SELF**
Reg No : **HL3901813667 / HYMH/BVI/H1944**
Barcode : **HEE125776**
INV : Hepatitis A Virus IgM Antibodies

Age : 24 Year | **Sex :** Male
Registered on : 14-08-2026 01:53 PM
Received on : 14-08-2026 05:48 PM
Reported on : 14-08-2026 08:22 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

HEPATITIS A VIRUS IGM ANTIBODIES

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HAV -IgM Method: ELISA	0.16	Index	0 - 0.90

Reference Range
Reference Range
Negative : < 0.90
Borderline : 0.9 - 1.1
Positive : >= 1.1

~~End of report~~

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REPORT

PT Name : **MRS.ARATI KANADE**
Ref By : **DR. VIJAY KANADE**
Reg No : **HL3601831721 / MH141**
Barcode : **HEN132319**
INV : Hepatitis A Virus IgG Antibodies (HAV IgG)

Age : 45 Year | **Sex :** Female
Registered on : 31-08-2026 09:43 AM
Received on : 31-08-2026 10:31 PM
Reported on : 02-09-2026 09:09 AM

SAMPLE COLLECTED AT :
DEVANSH CLINICAL LABORATORY, WASHIM

SAMPLE : Serum

HEPATITIS A VIRUS IGG ANTIBODIES (HAV IGG)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS A VIRUS IGG Method: ELISA	0.32	OD Ratio	0 - 0.90

Reference Range
Negative : < 0.90
Borderline : 0.9 - 1.1
Positive : >= 1.1

~~End of report~~

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REPORT

PT Name : **MR. ANIL CHAVAN**
Ref By : **DR. LIC**
Reg No : **HL4701830800 / MH005/MS103**
Barcode : **HEJ085144**
INV : Hemoglobin

Age : 42 Year | **Sex :** Male
Registered on : 30-08-2026 08:13 AM
Received on : 30-08-2026 09:27 AM
Reported on : 30-08-2026 10:12 AM

SAMPLE COLLECTED AT :
SPECTRUM DIAG

SAMPLE : EDTA Blood

HEMOGLOBIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Hemoglobin	13.9	g/dL	11 - 16

Interpreting test results

The numbers associated with your result are in grams per deciliter or g/dL. Hemoglobin reference ranges may vary depending on age and sex.

The high and low values of a normal test result, called the reference range, may differ depending on the laboratory and the methods used to conduct the test. For this reason, it is essential to look at the reference range listed on your test report.

An example of potential reference range, provided by the American Board of Internal Medicine, is listed in the following tables:

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REPORT

PT Name : **HETVI VYAS**
Ref By : **DR. SELF**
Reg No : **HL5401832635 / HYMH/KDV/H6569**
Barcode : **HEE125358**
INV : Glycated Haemoglobin (HbA1c)

Age : 25 Year | **Sex :** Female
Registered on : 02-09-2026 02:16 PM
Received on : 02-09-2026 05:20 PM
Reported on : 02-09-2026 07:06 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : EDTA Blood

GLYCATED HAEMOGLOBIN (HBA1C)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HBA1C Method: Fully Automated H.P.L.C.	5.4	%	0 - 6.5

BIOLOGICAL REFERENCE RANGES

Reference Range: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal 5.7% - 6.4% : Prediabetic >=6.5% : Diabetic	Below 6.5% : Good Control 6.5% - 7% : Fair Control 7.0% - 8% : Unsatisfactory Control >8% : Poor Control

Estimated Average Glucose : Method: CALCULATED	108.28	mg/dl	. - .

Reference Range

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Clinical significance :

Hemoglobin A1c (HbA1c) is a result of the nonenzymatic attachment of a hexose molecule to the N-terminal amino acid of the hemoglobin molecule. The attachment of the hexose molecule occurs continually over the entire life span of the erythrocyte and is dependent on blood glucose concentration and the duration of exposure of the erythrocyte to blood glucose. Therefore, the HbA1c level reflects the mean glucose concentration over the previous period (approximately 8-12 weeks, depending on the individual) and provides a much better indication of long-term glycemic control than blood and urinary glucose determinations. Diabetic patients with very high blood concentrations of glucose have from 2 to 3 times more HbA1c than normal individuals.

Please correlate with clinical conditions

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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125361**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:07 PM
Reported on : 02-09-2026 09:02 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : Fluoride Plasma-01

GLUCOSE F (BLOOD SUGAR FASTING)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
GLUCOSE, FASTING (F) Method: GOD-POD	76.0	mg/dl	74.0 - 110.0

Interpretation

- Depending on your treatment plan, you may check and record your blood sugar as many as four times a day or more often if you're taking insulin. Careful monitoring is the only way to make sure that your blood sugar level remains within your target range. People with type 2 diabetes who aren't taking insulin generally check their blood sugar much less frequently.

  People who receive insulin therapy also may choose to monitor their blood sugar levels with a continuous glucose monitor. Although this technology hasn't yet completely replaced the glucose meter, it can significantly reduce the number of fingersticks necessary to check blood sugar and provide important information about trends in blood sugar levels.

  Even with careful management, blood sugar levels can sometimes change unpredictably. With help from your diabetes treatment team, you'll learn how your blood sugar level changes in response to food, physical activity, medications, illness, alcohol, stress — and for women, fluctuations in hormone levels.

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **MR. RAM NAKSHATRA YADAV**
Ref By : **DR. SELF**
Reg No : **HL9201811060 / HYMH/KDV/H1886**
Barcode : **HEN173321**
INV : Globulin

Age : 68 Year | **Sex :** Male
Registered on : 11-08-2026 06:02 PM
Received on : 11-08-2026 06:28 PM
Reported on : 11-08-2026 10:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

GLOBULIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
GLOBULIN Method: CALCULATED	**4.02**	gm/dL	2.5 - 3.4

~~End of report~~

Pallavi.

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REPORT

PT Name : **MRS.MOHANBAI JAIN**
Ref By : **DR. PRATHAMESH HOSPITAL**
Reg No : **HL2501830340 / MH372**
Barcode : **HEN121162**
INV : GGT (Gamma Glutamyl Transferase)

Age : 82 Year | **Sex :** Female
Registered on : 29-08-2026 03:28 PM
Received on : 29-08-2026 06:24 PM
Reported on : 29-08-2026 08:30 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

GGT (GAMMA GLUTAMYL TRANSFERASE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Gamma GT Method: G glutamyl carboxy nitroanilide	**95.2**	U/L	5 - 55

Comments :

- The liver contains the highest levels of GGT, while the blood and some other organs contain minimal quantities.
- High levels of GGT in the blood could indicate that the enzyme is leaking out of the liver cells and into the blood, suggesting damage to the liver or bile ducts.
- A typical range for GGT levels in adults and children is between 0 and 30 international units per liter (IU/L). Newborn infants will have significantly higher levels right after birth.
- A person with a normal GGT test probably does not have liver disease. When someone who is in treatment for alcohol use disorder has a normal reading, this likely means that they have not had an alcoholic drink recently.
- GGT levels will rise according to the amount of liver damage a person has.

~~End of report~~

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REPORT

PT Name : **MRINDI KAMBLE**
Ref By : **DR. H1944**
Reg No : **HL4401830996 / HYMH/BVI/H1944**
Barcode : **HEN173898**
INV : FT4 (Thyroxine- Free)

Age : 20 Year | **Sex :** Female
Registered on : 30-08-2026 11:59 AM
Received on : 30-08-2026 06:02 PM
Reported on : 30-08-2026 08:45 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

FT4 (THYROXINE- FREE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Thyroxine - Free (FT4) Method: CLIA	1.15	ng/dL	0.89 - 1.76 Adults (21 - 87 Yrs) Pregnancy 0.7 - 2.0 : First Trimester 0.5 - 1.6 : 2nd and 3rd Tri

~~End of report~~

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REPORT

PT Name : **MRINDI KAMBLE**
Ref By : **DR. H1944**
Reg No : **HL4401830996 / HYMH/BVI/H1944**
Barcode : **HEN173898**
INV : FT3 (Trilodothyronine- Free)

Age : 20 Year | **Sex :** Female
Registered on : 30-08-2026 11:59 AM
Received on : 30-08-2026 06:02 PM
Reported on : 30-08-2026 08:45 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

FT3 (TRILODOTHYRONINE- FREE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Trilodothyronine Free (FT3) Method: CLIA	3.25	pg/mL	2.3 - 4.2 Pregnancy 2.0 - 3.8

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REPORT

PT Name	: **MR. ANAND PRAKASH PANDEY**	**Age :** 64 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 01-09-2026 04:01 PM	
Reg No	: **HL9801832602 / HYMH/KDV/H1886**	**Received on** : 01-09-2026 06:04 PM	
Barcode	: **HEN173358**	**Reported on** : 01-09-2026 07:29 PM	**SAMPLE :** EDTA Blood
INV	: Erythrocyte Sedimentation Rate (ESR)		

ERYTHROCYTE SEDIMENTATION RATE (ESR)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Erythrocyte sedimentation rate (ESR) Method: Wintrobe's	15	mm/Hour	0 - 20

NOTE

There are multiple causes of a high ESR test result. Some common conditions associated with a higher rate include:

- advanced age
- pregnancy
- anemia
- kidney disease
- obesity
- thyroid disease
- certain types of cancer, including some types of lymphoma and multiple myeloma

~~End of report~~

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REPORT

PT Name : **SANA**
Ref By : **DR. SELF**
Reg No : **HL9701832573 / RAJ41**
Barcode : **HEL002901**
INV : Basic Fever Panel

Age : 30 Year | **Sex :** Female
Registered on : 31-08-2026 11:40 PM
Received on : 01-09-2026 04:10 AM
Reported on : 01-09-2026 07:17 AM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

COMPLETE BLOOD COUNT (CBC)+ESR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE	METHOD
Hemoglobin	**11.0**	g/dL	12.5 - 14.5	SLS-Hemoglobin
Total Red Blood Cell Count	4.18	10^6/uL	3.82 - 4.8	Electrical Impedance
Hematocrit (HCT)	**31.9**	%	36 - 46	Calculated
Total Leucocytes Count	5.62	10^3/uL	4 - 10	Electrical Impedance
Neutrophils Percentage	63.3	%	40 - 80	Flow Cytometry/LM
Lymphocyte Percentage	34.2	%	20 - 40	Flow Cytometry/LM
Eosinophils Percentage	2.2	%	1 - 6	Flow Cytometry/LM
Monocytes Percentage	**0.1**	%	2 - 10	Flow Cytometry/LM
Basophils Percentage	0.2	%	0.0 - 1.0	Flow Cytometry/LM
Neutrophils-Absolute Count	3.56	10^3/uL	1.8 - 7.8	Calculated
Lymphocytes-Absolute Count	1.92	10^3/uL	0.8 - 4.8	Calculated
Eosinophil-Absolute Count	0.12	10^3/uL	0.0 - 0.50	Calculated
Monocyte- Absolute Count	**0.01**	10^3/uL	0.50 - 1.00	Calculated
Basophils-Absolute Count	0.01	10^3/uL	0.0 - 0.20	Calculated
Mean Corpuscular Volume (MCV) Method: RBC HISTOGRAM	**76.32**	fL	80 - 96	Calculated
Mean Corpuscular Hemoglobin (MCH)	**26.32**	pg	27 - 32	Calculated
Mean Corpuscular Hemoglobin Concentration (MCHC)	34.48	g/dL	31.52 - 35.5	Calculated
Red Cell Distribution Width (RDW-CV)	15.0	%	12 - 15	Calculated
Platelet Count	232	10^3/uL	150 - 410	Electrical Impedance
Mean Platelet Volume (MPV)	**11.6**	fL	6 - 11	Calculated
Platelet haematocrit (PCT)	0.269	%	0.1 - 0.28	Calculated
Platelet Distribution Width (PDW)	15.6	fL	15 - 18	Calculated
Erythrocyte Sedimentation Rate (ESR)	18	mm at end of Ist hr	00 - 20	Westergren

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SAURABH KOTHARI**
Ref By : **DR. SELF**
Reg No : **HL3301832634 / HYMH/KDV/H6569**
Barcode : **HEE125360**
INV : HY FITNESS SMART 3

Age : 40 Year | **Sex :** Male
Registered on : 02-09-2026 02:12 PM
Received on : 02-09-2026 05:31 PM
Reported on : 02-09-2026 06:10 PM

SAMPLE COLLECTED AT :
SHREE LAB

SAMPLE : EDTA Blood

COMPLETE BLOOD COUNT (CBC)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE	METHOD
Hemoglobin	13.5	g/dL	13 - 18	SLS-Hemoglobin
Total Red Blood Cell Count	4.65	10^6/uL	4.0 - 5.5	Electrical Impedance
Hematocrit (HCT)	44.0	%	33 - 57	Calculated
Total Leucocytes Count	6.32	10^3/uL	4 - 10	Electrical Impedance
Neutrophils Percentage	68.3	%	40 - 80	Flow Cytometry/LM
Lymphocyte Percentage	22.3	%	20 - 40	Flow Cytometry/LM
Eosinophils Percentage	3.9	%	1 - 6	Flow Cytometry/LM
Monocytes Percentage	5.2	%	2 - 10	Flow Cytometry/LM
Basophils Percentage	0.3	%	0.0 - 1.0	Flow Cytometry/LM
Neutrophils-Absolute Count	4.31	10^3/uL	1.8 - 7.8	Calculated
Lymphocytes-Absolute Count	1.41	10^3/uL	0.8 - 4.8	Calculated
Eosinophils-Absolute Count	0.25	10^3/uL	0.0 - 0.50	Calculated
Monocyte-Absolute Count	**0.33**	10^3/uL	0.50 - 1.00	Calculated
Basophils-Absolute Count	0.02	10^3/uL	0.0 - 0.20	Calculated
Mean Corpuscular Volume (MCV)	94.62	fL	80 - 96	Calculated
Mean Corpuscular Hemoglobin (MCH)	29.03	pg	27.5 - 32.2	Calculated
Mean Corpuscular Hemoglobin Concentration (MCHC)	30.68	g/dL	29.4 - 34.5	Calculated
Red Cell Distribution Width (RDW-CV)	13.4	%	12 - 15	Calculated
Platelet Count	295	10^3/uL	150 - 410	Electrical Impedance
Mean Platelet Volume (MPV)	9.7	fL	6 - 11	Calculated
Platelet haematocrit (PCT)	**0.286**	%	0.1 - 0.28	Calculated
Platelet Distribution Width (PDW)	**11.2**	fL	15 - 18	Calculated
Platelet larger cell Count (PLCC)	66	10^3/uL	13 - 126	Westergren
Platelet larger cell ratio (PLCR)	22.5	%	13 - 43	

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REPORT

PT Name : **SANA**
Ref By : **DR. SELF**
Reg No : **HL9701832573 / RAJ41**
Barcode : **HEL002901**
INV : Basic Fever Panel

Age : 30 Year | **Sex :** Female
Registered on : 31-08-2026 11:40 PM
Received on : 01-09-2026 04:10 AM
Reported on : 01-09-2026 07:17 AM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

COMPLETE BLOOD COUNT (CBC)+ESR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE	METHOD
Hemoglobin	**11.0**	g/dL	12.5 - 14.5	SLS-Hemoglobin
Total Red Blood Cell Count	4.18	10^6/uL	3.82 - 4.8	Electrical Impedance
Hematocrit (HCT)	**31.9**	%	36 - 46	Calculated
Total Leucocytes Count	5.62	10^3/uL	4 - 10	Electrical Impedance
Neutrophils Percentage	63.3	%	40 - 80	Flow Cytometry/LM
Lymphocyte Percentage	34.2	%	20 - 40	Flow Cytometry/LM
Eosinophils Percentage	2.2	%	1 - 6	Flow Cytometry/LM
Monocytes Percentage	**0.1**	%	2 - 10	Flow Cytometry/LM
Basophils Percentage	0.2	%	0.0 - 1.0	Flow Cytometry/LM
Neutrophils-Absolute Count	3.56	10^3/uL	1.8 - 7.8	Calculated
Lymphocytes-Absolute Count	1.92	10^3/uL	0.8 - 4.8	Calculated
Eosinophil-Absolute Count	0.12	10^3/uL	0.0 - 0.50	Calculated
Monocyte- Absolute Count	**0.01**	10^3/uL	0.50 - 1.00	Calculated
Basophils-Absolute Count	0.01	10^3/uL	0.0 - 0.20	Calculated
Mean Corpuscular Volume (MCV) Method: RBC HISTOGRAM	**76.32**	fL	80 - 96	Calculated
Mean Corpuscular Hemoglobin (MCH)	**26.32**	pg	27 - 32	Calculated
Mean Corpuscular Hemoglobin Concentration (MCHC)	34.48	g/dL	31.52 - 35.5	Calculated
Red Cell Distribution Width (RDW-CV)	15.0	%	12 - 15	Calculated
Platelet Count	232	10^3/uL	150 - 410	Electrical Impedance
Mean Platelet Volume (MPV)	**11.6**	fL	6 - 11	Calculated
Platelet haematocrit (PCT)	0.269	%	0.1 - 0.28	Calculated
Platelet Distribution Width (PDW)	15.6	fL	15 - 18	Calculated
Erythrocyte Sedimentation Rate (ESR)	18	mm at end of Ist hr	00 - 20	Westergren

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MR.SHAMSH ALAM**
Ref By : **DR. SELF**
Reg No : **HL2001827149 / RAJ41/RAY10**
Barcode : **HEL095818**
INV : Cholesterol LDL

Age : 45 Year | **Sex :** Male
Registered on : 26-08-2026 12:52 PM
Received on : 26-08-2026 05:05 PM
Reported on : 26-08-2026 07:03 PM

SAMPLE COLLECTED AT :
INDIAN CLINICAL LAB

SAMPLE : Serum

CHOLESTEROL LDL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LDL- Cholesterol Method: (PHOTOMETRY)	**113.6**	mg/dl	50 - 100 Desirable:100-129 Borderline High: 130-159 High: >160

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **TARUNNAM**
Ref By : **DR. SELF**
Reg No : **HL2201832577 / RAJ41**
Barcode : **HEL002911**
INV : Anti Cyclic Citrullinated Peptide Antibody (Anti-CCP)

Age : 45 Year | **Sex :** Female
Registered on : 31-08-2026 11:47 PM
Received on : 01-09-2026 04:10 AM
Reported on : 01-09-2026 06:01 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

ANTI CYCLIC CITRULLINATED PEPTIDE ANTIBODY (ANTI-CCP)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Anti CCP Method: CLIA	13.6	U/ml	0 - 20

Reference Range
20 Units or less: Negative
21-39 Units: Weak positive
40-59 Units: Moderate positive
60 Units or Greater: Strong positive

Clinical significance :
Rheumatoid arthritis (RA) is a systemic autoimmune disease characterized by chronic joint inflammation that ultimately leads to joint destruction. RA affects approximately 1% of the world's population. The diagnosis of RA is established primarily on clinical criteria and serologic findings. Historically, rheumatoid factor (RF), which is an antibody specific for the Fc portion of human IgG, has been considered a marker for RA. RF is, in fact, one of the diagnostic criteria for RA that was established by the American College of Rheumatology.(1) Although 50% to 90% of patients with RA are RF-positive, the specificity of the RF test is known to be relatively poor. RF is found in many patients with other autoimmune diseases, infectious diseases and some healthy individuals. Consequently, a search for better diagnostic markers, with improved specificity for RA, ensued. Antiperinuclear factor (APF) and antikeratin antibodies (AKA), identified by immunofluorescence, were found to have a specificity of close to 90% for RA, but testing for these autoantibodies has never become popular. It was subsequently determined that APF and AKA react with the same antigen, specifically a citrullinated form of filaggrin (citrulline is an unusual amino acid formed by posttranslational modification of arginine residues by the enzyme peptidyl arginine deaminase).(2) Recombinant filaggrin fragments, after enzymatic deamination in vitro, react with autoantibodies in RA sera. Synthetic cyclic citrullinated peptide (CCP) variants also react with anti-filaggrin autoantibodies and serve as the substrate for detecting anti-CCP antibodies serologically. Most studies of anti-CCP antibodies demonstrated that these autoantibodies have much improved specificity for RA compared to RF.(3)

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name	: **SAURABH KOTHARI**	**Age :** 40 Year | **Sex :** Male		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. SELF**	**Registered on**	: 02-09-2026 02:12 PM	SHREE LAB	
Reg No	: **HL3301832634 / HYMH/KDV/H6569**	**Received on**	: 02-09-2026 05:15 PM		
Barcode	: **HEE125362**	**Reported on**	: 02-09-2026 10:48 PM		
INV	: HY FITNESS SMART 3			**SAMPLE :** Serum	

C REACTIVE PROTEIN (CRP) QUANTITATIVE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
CRP Method: (Immunoturbidimetry)	**10.3**	mg/dL	0 - 6.0

Comments

CRP elevations are nonspecific and may be useful for the detection of systemic inflammatory processes; to assess treatment of bacterial infections with antibiotics; to detect intrauterine infections with concomitant premature amniorrhexis; to differentiate between active and inactive forms of disease with concurrent infection, eg, in patients suffering from systemic lupus erythematosus or colitis ulcerosa; to therapeutically monitor rheumatic disease and assess anti-inflammatory therapy; to determine the presence of postoperative complications at an early stage, such as infected wounds, thrombosis, and pneumonia; and to distinguish between infection and bone marrow rejection. Postoperative monitoring of CRP levels of patients can aid in the recognition of unexpected complications (persisting high or increasing levels).

Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. It also allows judgments about the disease genesis. Persistence of a high serum CRP concentration is usually a grave prognostic sign that generally indicates the presence of an uncontrolled infection.,

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name	: **MRS.PRIYA MALI8**	**Age :** 42 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. NEETI UPASANI**	**Registered on :** 31-08-2026 05:02 PM	
Reg No	: **HL4501832446 / MH466**	**Received on** : 31-08-2026 05:11 PM	
Barcode	: **HEL067129**	**Reported on** : 31-08-2026 07:48 PM	
INV	: BUN (Blood Urea Nitrogen)		**SAMPLE :** Serum

BUN (BLOOD UREA NITROGEN)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
BUN (Blood Urea Nitrogen) Method: CALCULATED	8.71	mg/dl	7 - 25

~~End of report~~

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REPORT

PT Name	: **MRS MADHAVI**	**Age :** 49 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 31-08-2026 04:56 PM	
Reg No	: **HL3101832432 / RAJ53**	**Received on** : 31-08-2026 05:13 PM	
Barcode	: **HEN121673**	**Reported on** : 31-08-2026 07:29 PM	
INV	: Bilirubin (Total + Direct + Indirect)		**SAMPLE :** Serum

BILIRUBIN (TOTAL + DIRECT + INDIRECT)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
BILIRUBIN - TOTAL Method: DIAZO	0.37	mg/dl	0 - 1.2
BILIRUBIN -DIRECT Method: DIAZO	0.15	mg/dl	0 - 0.4
BILIRUBIN (INDIRECT) Method: CALCULATED	0.22	mg/dl	0 - 0.9

Clinical significance :

Bilirubin is one of the most commonly used tests to assess liver function. Approximately 85% of the total bilirubin produced is derived from the heme moiety of hemoglobin, while the remaining 15% is produced from the red blood cell precursors destroyed in the bone marrow and from the catabolism of other heme-containing proteins. After production in peripheral tissues, bilirubin is rapidly taken up by hepatocytes where it is conjugated with glucuronic acid to produce mono- and diglucuronide, which are excreted in the bile.

A number of inherited and acquired diseases affect 1 or more of the steps involved in the production, uptake, storage, metabolism, and excretion of bilirubin. Bilirubinemia is a frequent and direct result of these disturbances.

Jaundice can occur as a result of problems at each step in the metabolic pathway. Disorders may be classified as those due to: increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice).

The most commonly occurring form of unconjugated hyperbilirubinemia is that seen in newborns and referred to as physiological jaundice. Elevated unconjugated bilirubin in the neonatal period may result in brain damage (kernicterus). Treatment options are phototherapy and, if severe, exchange transfusion.

The rare genetic disorders, Crigler-Najjar syndromes type I and type II, are caused by a low or absent activity of bilirubin UDP-glucuronyl-transferase. In type I, the enzyme activity is totally absent, the excretion rate of bilirubin is greatly reduced and the serum concentration of unconjugated bilirubin is greatly increased. Patients with this disease may die in infancy owing to the development of kernicterus.

The increased production of bilirubin, that accompanies the premature breakdown of erythrocytes and ineffective erythropoiesis, results in hyperbilirubinemia in the absence of any liver abnormality.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
Negative	Indicates absence of HCG in urine.

NOTE:

1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

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REPORT

PT Name	: **MRS. NAUSHEEN SHAIKH**	**Age :** 26 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 15-08-2026 11:48 AM	
Reg No	: **HL1101814476 / MH537**	**Received on** : 15-08-2026 06:20 PM	
Barcode	: **HEE152025**	**Reported on** : 15-08-2026 08:45 PM	**SAMPLE :** Urine
INV	: Pregnancy Test Urine		

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .

INTERPRETATION:

RESULT (INDEX)	REMARKS
Positive	Indicates presence of HCG in urine.
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1.Positive results in urine are seen in pregnancy in or after 1st week of missed period.
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4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

· To establish presumptive diagnosis of pregnancy

~~End of report~~

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REPORT

PT Name : **FATIMA BI TAHIR SAYYED**
Ref By : **DR. NIDA ANSARI**
Reg No : **HL1501832477 / MH621**
Barcode : **HEI085199**
INV : Prolactin (PRL)

Age : 26 Year | **Sex :** Female
Registered on : 31-08-2026 05:37 PM
Received on : 01-09-2026 05:53 PM
Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL) Method: CLIA	13.0	ng/ml	2.1 - 17.7

REFERENCE

Nonpregnant	2.8–29.2
Pregnant	9.7–208.5
Postmenopausal	1.8–20.3

Clinical significance :

Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary1 under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.2 Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.3 It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.4 Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~

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REPORT

PT Name : **ANIRUDDHA SATKAR**
Ref By : **DR. MAULI LAB**
Reg No : **HL5401831575 / MH244**
Barcode : **HEI047417**
INV : Protein - Total

Age : 28 Year | **Sex :** Male
Registered on : 30-08-2026 08:39 PM
Received on : 31-08-2026 09:01 AM
Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :
HORMOCARE

SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **MRS. NISHA MULE**
Ref By : **DR. SELF**
Reg No : **HL9401830688 / MH116**
Barcode : **HEL148767**
INV : Reticulocyte Count

Age : 30 Year | **Sex :** Female
Registered on : 29-08-2026 10:44 PM
Received on : 30-08-2026 03:33 AM
Reported on : 30-08-2026 12:54 PM

SAMPLE COLLECTED AT :
GANESH PANCHAL

SAMPLE : EDTA Blood

RETICULOCYTE COUNT

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RETICULOCYTE COUNT Method: microscopy	0.7	%	0.5 - 2.0

INTERPRETATION

Results must be interpreted carefully and along with results of other tests, such as an RBC count, hemoglobin (Hb), hematocrit (Hct), or a full CBC. In general, the reticulocyte count (absolute number or percentage) reflects recent bone marrow activity.

The results may indicate whether a disease or condition is present that is generating an increased demand for new RBCs and whether the bone marrow can respond to the extra requirement. Occasionally, results may indicate the overproduction of RBCs.

~~End of report~~

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REPORT

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REPORT

PT Name : **MRS. NISHA MULE**
Ref By : **DR. SELF**
Reg No : **HL9401830688 / MH116**
Barcode : **HEL148767**
INV : Reticulocyte Count

Age : 30 Year | **Sex :** Female
Registered on : 29-08-2026 10:44 PM
Received on : 30-08-2026 03:33 AM
Reported on : 30-08-2026 12:54 PM

SAMPLE COLLECTED AT :
GANESH PANCHAL

SAMPLE : EDTA Blood

RETICULOCYTE COUNT

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RETICULOCYTE COUNT Method: microscopy	0.7	%	0.5 - 2.0

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REPORT

PT Name : **MRS. NISHA MULE**
Ref By : **DR. SELF**
Reg No : **HL9401830688 / MH116**
Barcode : **HEL148767**
INV : Reticulocyte Count

Age : 30 Year | **Sex :** Female
Registered on : 29-08-2026 10:44 PM
Received on : 30-08-2026 03:33 AM
Reported on : 30-08-2026 12:54 PM

SAMPLE COLLECTED AT :
GANESH PANCHAL

SAMPLE : EDTA Blood

RETICULOCYTE COUNT

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RETICULOCYTE COUNT Method: microscopy	0.7	%	0.5 - 2.0

INTERPRETATION

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REPORT

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Ref By : **DR. SELF**
Reg No : **HL9401830688 / MH116**
Barcode : **HEL148767**
INV : Reticulocyte Count

Age : 30 Year | **Sex :** Female
Registered on : 29-08-2026 10:44 PM
Received on : 30-08-2026 03:33 AM
Reported on : 30-08-2026 12:54 PM

SAMPLE COLLECTED AT :
GANESH PANCHAL

SAMPLE : EDTA Blood

RETICULOCYTE COUNT

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RETICULOCYTE COUNT Method: microscopy	0.7	%	0.5 - 2.0

INTERPRETATION

Results must be interpreted carefully and along with results of other tests, such as an RBC count, hemoglobin (Hb), hematocrit (Hct), or a full CBC. In general, the reticulocyte count (absolute number or percentage) reflects recent bone marrow activity.

The results may indicate whether a disease or condition is present that is generating an increased demand for new RBCs and whether the bone marrow can respond to the extra requirement. Occasionally, results may indicate the overproduction of RBCs.

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REPORT

PT Name : **MRS. NISHA MULE**
Ref By : **DR. SELF**
Reg No : **HL9401830688 / MH116**
Barcode : **HEL148767**
INV : Reticulocyte Count

Age : 30 Year | **Sex :** Female
Registered on : 29-08-2026 10:44 PM
Received on : 30-08-2026 03:33 AM
Reported on : 30-08-2026 12:54 PM

SAMPLE COLLECTED AT :
GANESH PANCHAL

SAMPLE : EDTA Blood

RETICULOCYTE COUNT

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RETICULOCYTE COUNT Method: microscopy	0.7	%	0.5 - 2.0

INTERPRETATION

Results must be interpreted carefully and along with results of other tests, such as an RBC count, hemoglobin (Hb), hematocrit (Hct), or a full CBC. In general, the reticulocyte count (absolute number or percentage) reflects recent bone marrow activity.

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REPORT

PT Name : **MRS PRIYANKA ARKADE**
Ref By : **DR. SELF**
Reg No : **HL8801832560 / MH601**
Barcode : **HEI033680**
INV : HY FIT3 +RA

Age : 29 Year | **Sex :** Female
Registered on : 31-08-2026 07:49 PM
Received on : 01-09-2026 06:39 PM
Reported on : 01-09-2026 07:15 PM

SAMPLE COLLECTED AT :
RUPESH DIGNOSTIC

SAMPLE : Serum

RHEUMATOID FACTOR, RA TEST

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RHEUMATOID FACTOR, RA	10.2	IU/mL	1.0 - 20.0

Method: Immunoturbidimetry

What is rheumatoid factor

Rheumatoid factor is an antibody that is measurable in the blood with a routine blood test. Rheumatoid factor is actually an antibody that can bind to other antibodies. Antibodies are normal proteins in our blood that are important parts of our immune system. Rheumatoid factor is an antibody that is not usually present in the normal individual. Because rheumatoid factor antibody binds to normal antibodies, it can be generally referred to as an autoantibody. Health care professionals use the rheumatoid factor test to assist in the diagnosis of rheumatoid arthritis.

Arthritis refers to more than 100 conditions that affect the musculoskeletal system. The joints are the parts of the body where bones meet each other. When arthritis is present, the joints may become inflamed, stiff, red, and painful. Damage from RA may occur in tissues surrounding the joints including the tendons, ligaments, and muscles. RA is one type of arthritis classified as 'systemic,' meaning it can areas throughout the body. In some patients, symptoms may extend to the skin and eyes, and internal organs, including the liver, kidneys, heart, and lungs.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MRS PRIYANKA ARKADE**
Ref By : **DR. SELF**
Reg No : **HL8801832560 / MH601**
Barcode : **HEI033680**
INV : HY FIT3 +RA

Age : 29 Year | **Sex :** Female
Registered on : 31-08-2026 07:49 PM
Received on : 01-09-2026 06:39 PM
Reported on : 01-09-2026 07:15 PM

SAMPLE COLLECTED AT :
RUPESH DIGNOSTIC

SAMPLE : Serum

RHEUMATOID FACTOR, RA TEST

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
RHEUMATOID FACTOR, RA	10.2	IU/mL	1.0 - 20.0

Method: Immunoturbidimetry

What is rheumatoid factor

Rheumatoid factor is an antibody that is measurable in the blood with a routine blood test. Rheumatoid factor is actually an antibody that can bind to other antibodies. Antibodies are normal proteins in our blood that are important parts of our immune system. Rheumatoid factor is an antibody that is not usually present in the normal individual. Because rheumatoid factor antibody binds to normal antibodies, it can be generally referred to as an autoantibody. Health care professionals use the rheumatoid factor test to assist in the diagnosis of rheumatoid arthritis.

Arthritis refers to more than 100 conditions that affect the musculoskeletal system. The joints are the parts of the body where bones meet each other. When arthritis is present, the joints may become inflamed, stiff, red, and painful. Damage from RA may occur in tissues surrounding the joints including the tendons, ligaments, and muscles. RA is one type of arthritis classified as 'systemic,' meaning it can areas throughout the body. In some patients, symptoms may extend to the skin and eyes, and internal organs, including the liver, kidneys, heart, and lungs.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **MR.ABDUL LATEEF**
Ref By : **DR. SELF**
Reg No : **HL8201832322 / RAJ41/RAY10**
Barcode : **HEL095766**
INV : SGOT ; Aspartate Aminotransferase (AST)

Age : 66 Year | **Sex :** Male
Registered on : 31-08-2026 04:00 PM
Received on : 31-08-2026 06:21 PM
Reported on : 31-08-2026 08:18 PM

SAMPLE COLLECTED AT :
INDIAN CLINICAL LAB

SAMPLE : Serum

SGOT ; ASPARTATE AMINOTRANSFERASE (AST)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Aspartate Transaminase (AST/SGOT) Method: PHOTOMETRY	**64.98**	U/L	0 - 35

Clinical significance :

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **ALISHA ANSARI**
Ref By : **DR. TABREZ**
Reg No : **HL9401832467 / MH621**
Barcode : **HEI085201**
INV : SGPT ; ALANINE AMINOTRANSFERASE (ALT)

Age : 18 Year | **Sex :** Female
Registered on : 31-08-2026 05:23 PM
Received on : 31-08-2026 06:01 PM
Reported on : 31-08-2026 07:09 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

SGPT ; ALANINE AMINOTRANSFERASE (ALT)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SGPT	11.89	U/L	0 - 34

Method: PHOTOMETRY

Clinical significance :

Alanine aminotransferase (ALT) is present primarily in liver cells. In viral hepatitis and other forms of liver disease associated with hepatic necrosis, serum ALT is elevated even before the clinical signs and symptoms of the disease appear. Although serum levels of both aspartate aminotransferase (AST) and ALT become elevated whenever disease processes affect liver cell integrity, ALT is a more liver-specific enzyme. Serum elevations of ALT are rarely observed in conditions other than parenchymal liver disease. Moreover, the elevation of ALT activity persists longer than does AST activity

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : **MS LAXMI YADAV**
Ref By : **DR. SHIVAM PATHOLOGY**
Reg No : **HL4701831623 / MH607**
Barcode : **Heb008419**
INV : Sodium

Age : 19 Year | **Sex :** Female
Registered on : 31-08-2026 12:07 AM
Received on : 31-08-2026 11:04 AM
Reported on : 31-08-2026 03:25 PM

SAMPLE COLLECTED AT :
SHIVAM PATHOLOGY LAB

SAMPLE : Serum

SODIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SODIUM , SERUM Method: (Electrode)	**130.30**	mEq/L	133 - 146

~~End of report~~

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SODIUM

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SODIUM , SERUM Method: (Electrode)	**130.30**	mEq/L	133 - 146

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SODIUM , SERUM Method: (Electrode)	**130.30**	mEq/L	133 - 146

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